| BEGINNER LEVEL | TEST-E ONLY | 10–12 WEEKS | COMPLETE PCT |
|---|---|---|---|

# BEGINNER ANABOLIC CYCLE COMPLETE GUIDE

Your Safe, Science-Based Entry Into Hormonal Enhancement

*Test-Only Foundation • Injection Protocol • PCT • Blood Work*

| ROYAL FITNESS CLUB — ANABOLIC SERIES | Anabolic Series Vol. 1 • Beginner Edition • 2024 |
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*DISCLAIMER: This document is for educational purposes only. Anabolic steroids are controlled substances in many countries. Always consult a licensed physician before starting any hormonal protocol. The authors do not condone illegal use.*

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## ■ 1. Introduction & Who Is This Guide For

This guide is designed for healthy adult males (25+) who have at minimum 3–5 years of serious natural training experience and have maximised their natural genetic potential. A beginner cycle does NOT mean you are new to the gym — it means you are new to anabolic substances.

| Criteria | Minimum Requirement |
|---|---|
| Training Age | 3–5 years of consistent, structured training |
| Body Fat | Below 15% (ideally 10–12%) before starting |
| Age | Minimum 23–25 years (HPTA fully matured) |
| Natural Max | Close to or at natural genetic potential |
| Knowledge | Understand nutrition, training, and recovery fully |
| Blood Work | Pre-cycle bloods completed and reviewed by physician |
| PCT Meds | Nolvadex/Clomid on hand BEFORE starting |

■■ **IMPORTANT:** Never start a cycle without having your PCT medications already purchased and in hand. Never skip blood work.

## ■■ 2. Understanding Testosterone Esters

Testosterone is available in several ester forms that determine its half-life — how long it stays active in the bloodstream. For beginners, long-estered forms are preferred as they require less frequent injections and produce stable blood levels.

| Ester | Half-Life | Injection Freq. | Active Duration | Beginner Use |
|---|---|---|---|---|
| Testosterone Enanthate | 4.5 days | 2x/week | ~14 days | ■ Ideal |
| Testosterone Cypionate | 5 days | 2x/week | ~16 days | ■ Ideal |
| Testosterone Propionate | 0.8 days | EOD | ~3 days | ■ Too frequent |
| Sustanon 250 | Multi-ester | 2x/week | ~21 days | ■■ Less stable levels |
| Testosterone Undecanoate | 16.5 days | 1x/week | ~34 days | ■■ Overdose risk if misdosed |

## ■ Testosterone Dose-Response Guide

| Weekly Dose | Expected Effect | Aromatisation Risk | Recommended For |
|---|---|---|---|
| 200 mg/week | TRT-level, mild lean gains | Low | HRT patients |
| 250–300 mg/week | Beginner sweet spot, solid gains | Moderate | First cycle (recommended) |
| 400 mg/week | Noticeable muscle growth | Moderate–High | First cycle (aggressive) |
| 500 mg/week | Strong anabolic effect | High | 2nd cycle onwards |
| 600+ mg/week | Diminishing returns, high E2 risk | Very High | Intermediate only |

■ *Testosterone aromatises (converts) to oestradiol (E2) via the aromatase enzyme. High E2 causes water retention, gynecomastia, and mood issues. Always have an aromatase inhibitor (AI) on hand.*

## ■ 3. First Cycle Protocol — Testosterone Enanthate 10 Weeks

The classic beginner protocol: Test-E only, 300–400 mg/week, 10–12 weeks. This single-compound approach allows you to understand exactly how your body responds to exogenous testosterone before adding complexity.

| Compound | Dose | Frequency | Weeks | Purpose |
|---|---|---|---|---|
| Testosterone Enanthate | 150–200 mg | 2x per week (Mon/Thu) | 1–10 | Primary anabolic agent |
| Anastrozole (AI) | 0.5 mg | Every other day (as needed) | 1–10 | Oestrogen control |
| Aromasin (alternative AI) | 12.5 mg | Every other day (as needed) | 1–10 | Oestrogen control (preferred) |
| Nolvadex (SERM — PCT) | 40/40/20/20 mg | Daily | Weeks 12–15 | PCT — restart HPTA |
| Clomid (optional PCT) | 50/50/25/25 mg | Daily | Weeks 12–15 | Additional HPTA support |

## ■ 4. Week-by-Week Full Schedule

| Week | Test-E Dose | AI Protocol | Blood Work | Notes |
|---|---|---|---|---|
| 1 | 150 mg ×2 | Hold — monitor | — | Start low, assess sides |
| 2 | 150 mg ×2 | Hold — monitor | — | Track mood, libido, acne |
| 3 | 200 mg ×2 | 0.5mg Anastrozole if needed | — | Compound begins fully saturating |
| 4 | 200 mg ×2 | Continue if E2 sides present | Optional mid-cycle | Strength gains begin |
| 5 | 200 mg ×2 | Consistent AI dose | — | Peak anabolic environment |
| 6 | 200 mg ×2 | Consistent AI dose | — | Water retention peaks and stabilises |
| 7 | 200 mg ×2 | Consistent AI dose | — | Steady state — solid gains |
| 8 | 200 mg ×2 | Consistent AI dose | — | Assess body composition |
| 9 | 200 mg ×2 | Taper AI slightly | — | Prepare for cycle end |
| 10 | 200 mg ×2 | Last AI dose with last injection | — | Final week — last pin |
| 11 | — | No AI | End-of-cycle bloods | Clearance period — 2 weeks |
| 12 | — | No AI | — | PCT Week 1 begins: Nolva 40mg |
| 13 | — | — | — | PCT Week 2: Nolva 40mg |
| 14 | — | — | — | PCT Week 3: Nolva 20mg |
| 15 | — | — | Post-PCT bloods | PCT Week 4: Nolva 20mg — DONE |

## ■ 5. Injection Protocol — Sites, Technique & Sterility

Proper injection technique is non-negotiable for safety. Infections from improper technique can require hospitalisation. Follow every step meticulously.

| Injection Site | Needle Size | Volume Limit | Difficulty | Notes |
|---|---|---|---|---|
| Glute (ventrogluteal) | 23G 1.5 inch | 3 mL | Easy | Most beginner-friendly, fewest nerves/vessels |
| Glute (dorsogluteal) | 23G 1.5 inch | 3 mL | Medium | Upper outer quadrant only — nerve risk |
| Vastus lateralis (outer thigh) | 23G 1 inch | 2 mL | Easy | Good self-injection site |
| Deltoid (shoulder) | 25G 1 inch | 1 mL | Medium | Small volume only |
| Pectoral | 25G 1 inch | 1 mL | Difficult | Not recommended for beginners |

| Step | Action | Why |
|---|---|---|
| 1 | Wash hands thoroughly with soap for 30 seconds | Prevent contamination |
| 2 | Wipe vial top with fresh alcohol swab | Sterility |
| 3 | Draw air equal to dose, inject into vial | Prevents vacuum |
| 4 | Draw out oil + 10–20% extra, remove bubbles | Accurate dosing |
| 5 | Swap to new injection needle | Blunt drawing needle causes tissue damage |
| 6 | Wipe injection site with alcohol swab — let dry | Wet alcohol can cause stinging/infection |
| 7 | Insert needle at 90°, pull back plunger slightly | Confirm not in vein (no blood = safe) |
| 8 | Inject slowly over 20–30 seconds | Reduces post-injection pain (PIP) |
| 9 | Remove needle, apply gentle pressure, discard safely | Sterile disposal |

■■ **IMPORTANT:** NEVER re-use needles. Never inject if you see any cloudiness or particles in the oil. Post-injection pain (PIP) for 1–3 days is normal. Swelling, redness, and fever are signs of infection — seek medical attention immediately.

## 6. Side Effects — Identification & Management

| Side Effect | Cause | Symptoms | Prevention / Treatment |
|---|---|---|---|
| Gynecomastia | High oestradiol (E2) | Puffy/tender nipples, lump under nipple | AI (Anastrozole/Aromasin). SERM (Nolva) if lump appears |
| Water Retention | High E2, sodium retention | Bloating, puffy face/fingers, high BP | AI management, reduce sodium, increase water |
| Acne | DHT + sebaceous gland activity | Back acne, face acne | Benzoyl peroxide wash, topical retinoids, zinc |
| Hair Loss | DHT sensitivity (genetic) | Temple recession, thinning | Nizoral shampoo, topical minoxidil — if prone |
| Testicular Atrophy | LH/FSH suppression | Reduced testis size | Normal on cycle — reverses with proper PCT |
| High Blood Pressure | Water retention, increased RBC | Headaches, epistaxis | Manage E2, monitor BP weekly, reduce dose |
| Increased RBC/Haematocrit | Testosterone stimulates erythropoiesis | Thick blood, elevated haematocrit | Blood donation, hydration, monitor bloods |
| Mood Swings | E2 fluctuation, hormonal shifts | Irritability, anxiety, aggression | Stable AI dosing, consistent injection schedule |
| Libido Changes | E2 too high or too low | Low libido, ED | Dial in E2 level — sweet spot ~20–30 pg/mL |

## ■ 7. Blood Work — The Complete Testing Guide

Blood work is the single most important safety measure for any anabolic cycle. It is non-negotiable. Always test before, during, and after.

| Test | Pre-Cycle | Mid-Cycle (Wk 5–6) | End-Cycle (Wk 11) | Post-PCT (Wk 16) | Purpose / Reference Range |
|---|---|---|---|---|---|
| Total Testosterone | ■ | ■ | ■ | ■ | Natural baseline + recovery check |
| Free Testosterone | ■ | — | — | ■ | Bioavailable T |
| Oestradiol (E2 sensitive) | ■ | ■ | ■ | ■ | Target: 20–30 pg/mL on cycle |
| LH / FSH | ■ | — | — | ■ | Should recover to baseline post-PCT |
| SHBG | ■ | — | — | ■ | Affects free testosterone |
| Full Blood Count (FBC) | ■ | ■ | ■ | ■ | Check haematocrit <54% |
| Lipid Panel (LDL/HDL) | ■ | ■ | — | ■ | Steroids tank HDL |
| Liver Function (AST/ALT) | ■ | — | ■ | ■ | Injectable test — minimal liver stress |
| Kidney Function (eGFR/Creatinine) | ■ | — | ■ | ■ | Baseline function |
| Blood Pressure | ■ | Weekly | Weekly | ■ | Target: below 130/85 mmHg |
| Glucose / HbA1c | ■ | — | — | ■ | Insulin sensitivity marker |
| PSA (if 35+) | ■ | — | — | ■ | Prostate health |

## ■ 8. PCT — Full Post Cycle Therapy Protocol

PCT begins 2 weeks after the last injection of testosterone enanthate (to allow clearance). The goal is to restart natural testosterone production as quickly and completely as possible.

| PCT Week | Nolvadex | Clomid (optional) | Supplements | Goal |
|---|---|---|---|---|
| Week 1 (Wk 12) | 40 mg/day | 50 mg/day | Zinc 30mg, Vitamin D3 5000IU | Initiate LH/FSH release |
| Week 2 (Wk 13) | 40 mg/day | 50 mg/day | Continue supplements | Continued HPTA stimulation |
| Week 3 (Wk 14) | 20 mg/day | 25 mg/day | Add Ashwagandha KSM-66 | Taper — natural T rising |
| Week 4 (Wk 15) | 20 mg/day | 25 mg/day | Continue all supplements | Final taper — assess recovery |
| Week 5+ (optional) | 10 mg/day EOD | — | Continue supplements | If recovery slow, extend |
| Blood Work (Wk 16) | Stop all SERMs | — | Continue support supps | Confirm recovery — check LH/FSH/T |

## 9. Nutrition on Cycle — Maximising Your Gains

Anabolic steroids significantly increase nitrogen retention and protein synthesis. Your nutrition must be dialled in to take full advantage of this anabolic environment.

| Macro | Bulking (Test 300–400mg) | Cutting (Test 300mg) | Per kg of bodyweight | Notes |
|---|---|---|---|---|
| Protein | 220–260 g/day | 240–280 g/day | 2.2–3.0 g/kg | Higher requirement on cycle — repair demand elevated |
| Carbohydrates | 400–600 g/day | 150–250 g/day | 4–6 g/kg (bulk) | Primary fuel — drives strength and pumps |
| Fats | 80–120 g/day | 60–80 g/day | 0.8–1.2 g/kg | Hormone production, lipid health |
| Total Calories | Surplus +400–600 | Deficit -300–400 | Body weight dependent | Track weekly weight — aim +0.25–0.5 kg/week |
| Water | 4–6 L/day | 4–6 L/day | — | Testosterone increases blood viscosity — hydrate |
| Sodium | Moderate | Moderate-Low | — | Excess sodium worsens water retention with high E2 |

| Training Variable | Natural | On Cycle (Test 300–400mg) | Reason |
|---|---|---|---|
| Weekly Volume | 10–20 sets/muscle | 16–28 sets/muscle | Enhanced recovery capacity |
| Training Frequency | 2–3x/muscle/week | 3–4x/muscle/week | Faster muscle protein synthesis |
| Intensity | 85–90% 1RM max | 90–95% 1RM | CNS drive + strength increase |
| Recovery Time | 48–72 hrs/muscle | 24–36 hrs/muscle | Accelerated recovery |
| Deload | Every 4–6 weeks | Every 6–8 weeks | Reduced recovery need |
| Cardio | 2–3x/week | 3–4x/week | Offset cardiovascular impact of AAS |